

The Medication Audit

Nothing has to be wrong for a medication to stop being appropriate. Bodies change, and a prescription started for a good reason years ago can outlast its utility or safety if not monitored. This isn't about hunting for mistakes. It's the same review I run with every new patient: asking, of each medication, whether it still belongs. Keep these questions handy for a medication review, a doctor's visit, or a care plan meeting.

01 Build the Full List First

Before anything else, get one written list: every prescription, every over-the-counter drug, everything taken as needed, and every supplement, with dose, timing, purpose, prescriber, and the date each one was started. The Medication List, the companion sheet in this kit, is where that goes. Fill it out and bring it with you whenever medications are in the conversation.

The list itself is worth a second look, independent of any single medication on it. The risk of a dangerous interaction climbs with every drug added, even when each one individually still makes sense. If the list has grown long, that's reason enough on its own to ask whether anything can be dropped.

02 Who's Managing This?

Can your loved one say what each medication is and why they take it? Ask directly, don't assume. When that answer gets shaky, it's usually the first real sign medication management needs more support, whether from a caregiver or the community's staff. That support can take several forms: help filling a pill box, a timed dispenser, or having community staff administer medications directly. None of it is a loss of independence. It's the opposite: a missed dose, a dangerous side effect, or a fall from a medication mix-up costs your loved one far more independence than asking for help ever will.

03 Ask About the Beers Criteria

The Beers Criteria is the American Geriatrics Society's list of medications that deserve a second look in older adults. A plain-language version of the Beers Criteria lives at [HealthinAging.org](https://www.healthinaging.org). A medication's presence on the list isn't an automatic reason to stop it. For some people, a flagged medication is still the right choice. If your loved one is taking something on the list, don't stop it on your own: bring it to their doctor and ask whether a safer option exists, and if not, why the current one still makes sense.

04 Cross-Check the MAR

Request the medication administration record (MAR): the log of what's actually being given and when. Compare it against your list. Look for a drug on the MAR that isn't on your list, a dose that doesn't match what the doctor last ordered, or something marked discontinued that's still being given. Any of these goes to the community's nurse or med tech that day, not the next scheduled review.

05 Check the Schedule, Not Just the Dose

Most medications have some flexibility, commonly up to an hour before or after schedule; time-critical ones don't. If something genuinely is time-critical (a classic example is medication for Parkinson's disease), flag it directly on the Medication List: circle the name or write "time-critical" beside it, then check the MAR to confirm it's actually landing inside that tighter window, not the standard one.

How an order is written also matters: "three times a day" and "every 8 hours" aren't actually the same. Three times a day conventionally means with meals, while every 8 hours means true even spacing. Followed too literally, that difference can mean an unnecessary overnight wake-up on one order, or too long a gap on the other, for a medication that actually needed the even spacing. Ask the doctor which one each specific medication needs, and confirm the MAR matches that.

06 Ask About As-Needed (PRN) Medications

An order existing doesn't mean the medication is being given. For someone who can't reliably ask for help, that gap matters. For each PRN medication, ask what it's for, who decides to give it, and how often it's actually being used. If community staff are managing medications, ask to see the MAR: it should show every dose actually given, available for your review.

07 Question Every New or Standing Order Before You Sign

At admission, the community will often suggest standing "as-needed" orders (something for constipation, a cough, minor pain) so they're available without waiting on a doctor's call. Extra orders raise the risk of a prescribing cascade, one drug's side effect treated with another drug. Many communities also price by how many medications your loved one takes, so unused orders can push them into a higher tier. Consult with your loved one's doctor to make sure everything being suggested is something they recommend.

08 Confirm Medication Changes After a Hospital Stay or Doctor Visit

Transitions of care are some of the most common times when medication errors occur. Confirm any new or changed medication reached both the pharmacy and the community, and that it's showing correctly on the MAR if a community is managing administration. Confirm anything discontinued has actually stopped.

Schedule a visit with your loved one's primary care doctor as soon after the transition as you can. That's where someone with the full picture checks that the new medication list actually makes sense together: nothing duplicated, nothing conflicting, nothing left over from before the hospitalization that no longer belongs.

09 Set a Cadence

Run this whole review at least once a year, and anytime something changes: a fall, a new diagnosis, a hospitalization, or a new flag on the Beers Criteria list. A list that was right six months ago isn't a guarantee it's right today.

GO DEEPER

Chapter 25 of *The Long Road* goes deeper into the reasoning behind every question on this worksheet. The rest of the book covers what comes before and after it: whether it's time to make a move, how to raise the topic with your loved one, what care actually costs, how to choose well, and what to watch for once they've moved in. Find it at geriatriciansguide.com.

Get *The Long Road* →

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